pharmaniaga®

Zithrolide®

Azithromycin

ZITHROLIDE® POWDER FOR ORAL SUSPENSION 200MG/5ML

PRODUCT COMPOSITION

Each 5 ml prepared suspension contains 209.62 mg of Azithromycin Dihydrate equivalent to 200 mg of Azithromycin.

PRODUCT DESCRIPTION

Before reconstitution: White to off-white granules.

After reconstitution: Slightly translucent off-white suspension with tutti fruity flavour.

PHARMACODYNAMICS

General Properties

Pharmacotherapeutic group: Macrolides, ATC code J01FA.

Mode of action

Azithromycin is the first of a subclass of macrolide antibiotics, known as azalides, and is chemically different from erythromycin. Chemically it is derived by insertion of a nitrogen atom into the lactone ring of erythromycin A. The chemical name of azithromycin is 9-deoxy-9a-aza-9a-methyl-9a-ho-moerythromycin A. The molecular weight is 749.0.

Azithromycin binds to the 23S rRNA of the 50S ribosomal subunit. It blocks protein synthesis by inhibiting the transpeptidation/translocation step of protein synthesis and by inhibiting the assembly of the 50S ribosomal subunit.

Cardiac electrophysiology

Based on published data, QTc interval prolongation was studied in a randomized, placebo-controlled parallel trial in 116 healthy subjects who received either chloroquine (1000 mg) alone or in combination with azithromycin (500 mg, 1000 mg, and 1500 mg once daily). Co-administration of azithromycin increased the QTc interval in a dose- and concentration-dependent manner. In comparison to chloroquine alone, the maximum mean (95% upper confidence bound) increases in QTcF were 5 (10) ms, 7 (12) ms and 9 (14) ms with the co-administration of 500 mg, 1000 mg and 1500 mg azithromycin, respectively.

Mechanism of resistance

The two most frequently encountered mechanisms of resistance to macrolides, including azithromycin, are target modification (most often by methylation of 23S rRNA) and active efflux.

The occurrence of these resistance mechanisms varies from species to species and, within a species, the frequency of resistance varies by geographical location.

The most important ribosomal modification that determines reduced binding of macrolides is post-transcriptional (N6) -dimethylation of adenine at nucleotide A2058 (*Escherichia coli* numbering system) of the 23S rRNA by methylases encoded by *erm* (erythromycin ribosome methylase) genes. Ribosomal modifications often determine cross-resistance (MLSB phenotype) to other classes of antibiotics whose ribosomal binding sites overlap those of the macrolides: the lincosamides (including clindamycin), and the streptogramins B (which include, for example, the quinupristin component of quinupristin/dalfopristin). Different *erm* genes are present in different bacterial species, in particular streptococci and staphylococci. Susceptibility to macrolides can also be affected by less frequently encountered mutational changes in nucleotides A2058 and A2059, and at some other positions of 23S rRNA, or in the large subunit ribosomal proteins L4 and L22.

Efflux pumps occur in a number of species, including gram-negatives, such as *Haemophilus influenzae* (where they may determine intrinsically higher minimal inhibitory concentrations [MICs]) and staphylococci. In streptococci and enterococci, an efflux pump that recognizes 14- and 15-membered macrolides (which include, respectively, erythromycin and azithromycin) is encoded by *mef* (A) genes.

Antibacterial spectrum:

The prevalence of acquired resistance may vary geographically and with time for selected species, and local information on resistance is desirable, particularly when treating severe infections. As necessary, expert advice should be sought when the local prevalence of resistance is such that the utility of the agent in at least some types of infections is questionable.

Azithromycin demonstrates cross-resistance with erythromycin-resistant gram-positive isolates. As discussed above, some ribosomal modifications determine cross-resistance with other classes of antibiotics whose ribosomal binding sites overlap those of the macrolides: the lincosamides (including clindamycin), and the streptogramin B (which include, for example, the quinupristin component of quinupristin/dalfopristin). A decrease in macrolide susceptibility over time has been noted in particular in *Streptococcus pneumoniae* and *Staphylococcus aureus*, and has also been observed in viridans streptococci and *Streptococcus agalactiae*.

Organisms that are commonly susceptible to azithromycin include:

Aerobic and facultative gram-positive bacteria (erythromycin-susceptible isolates): *S. aureus, Streptococcus agalactiae,* S. pneumoniae,* Streptococcus pyogenes,** other β-hemolytic streptococci (Groups C, F, G) and viridans streptococci. Macrolide-resistant isolates are encountered relatively frequently among aerobic and facultative gram-positive bacteria, in particular among methicillin-resistant *S. aureus* (MRSA) and penicillin-resistant *S. pneumoniae* (PRSP).

Aerobic and facultative gram-negative bacteria: *Bordetella pertussis, Haemophilus ducreyi,* Haemophilus influenzae,* Haemophilus parainfluenzae,* Legionella pneumophila, Moraxella catarrhalis,** and *Neisseria gonorrhoeae.* Pseudomonas* spp. and most *Enterobacteriaceae* are inherently resistant to azithromycin, although azithromycin has been used to treat *Salmonella enterica* infections.

Anaerobes: *Clostridium perfringens, Peptostreptococcus* spp. and *Prevotella bivia*.

Other bacterial species: *Borrelia burgdorferi, Chlamydia trachomatis, Chlamydophila pneumoniae,**
Mycoplasma pneumoniae, Treponema pallidum,* and *Ureaplasma urealyticum*.

Opportunistic pathogens associated with HIV infection: Eukaryotic microorganisms *Pneumocystis jirovecii* and *Toxoplasma gondii*.

* The efficacy of azithromycin against the indicated species has been demonstrated in published clinical trials.

PHARMACOKINETICS

Absorption

Following oral administration in humans, azithromycin is widely distributed throughout the body; bioavailability is approximately 37%. The time taken to peak plasma levels is 2 to 3 hours.

Distribution

Based on published data of animal studies, high azithromycin concentrations have been observed in phagocytes. In experimental models, higher concentrations of azithromycin are released during active phagocytosis than from non-stimulated phagocytes. In animal models, this results in high concentrations of azithromycin being delivered to the site of infection.

Pharmacokinetic studies in humans have shown markedly higher azithromycin levels in tissues than in plasma (up to 50 times the maximum observed concentration in plasma), indicating that the drug is heavily tissue bound. Concentrations in target tissues, such as lung, tonsil and prostate, exceed the MIC_{90} for likely pathogens after a single dose of 500mg.

Elimination

Plasma terminal elimination half-life closely reflects the tissue depletion half-life of 2 to 4 days. Approximately 12% of an intravenously administered dose is excreted in the urine over 3 days as the parent drug, the majority in the first 24 hours. Biliary excretion of azithromycin is a major route of elimination for unchanged drug following oral administration. Very high concentrations of unchanged drug have been found in human bile, together with 10 metabolites, formed by N- and O-demethylation, hydroxylation of the desosamine and aglycone rings, and cleavage of the cladinose conjugate. Comparison of HPLC and microbiological assays in tissues suggests that metabolites play no part in the microbiological activity of azithromycin.

Special Populations:

Elderly

In elderly volunteers (>65years), slightly higher AUC values were seen after a 5-day regimen than in young volunteers (<40 years), but these are not considered clinically significant, and hence no dose adjustment is recommended.

Renal impairment

The pharmacokinetics of azithromycin in subjects with mild to moderate renal impairment (GFR 10-80 ml/min) were not affected following a single 1-gram dose of immediate-release azithromycin. Statistically significant difference in AUC_{0-120} (8.8 μg.h/ml vs. 11.7 μg.h/ml), C_{max} (1.0 μg/ml vs. 1.6 μg/ml) and CLr (2.3 ml/min/kg vs. 0.2 ml/min/kg) were observed between the group with severe renal impairment (GFR <10 ml/min) and the group with normal renal function.

Hepatic Impairment

In patients with mild (Class A) to moderate (Class B) hepatic impairment, there is no evidence of a marked change in serum pharmacokinetics of azithromycin compared to those with normal hepatic function. In these patients, urinary clearance of azithromycin appears to increase, perhaps to compensate for reduced hepatic clearance.

INDICATIONS

Azithromycin is indicated for infections caused by susceptible organisms; in lower respiratory tract infections including bronchitis and pneumonia, in skin and soft tissue infections, in acute otitis media and in upper respiratory tract infections including sinusitis and pharyngitis/tonsillitis.(Penicillin is the usual drug of choice in the treatment of Streptococcus pyogenes pharyngitis, including the prophylaxis of rheumatic fever. Azithromycin is generally effective in the eradication of streptococci from the oropharynx; however, data establishing the efficacy of azithromycin and the subsequent prevention of rheumatic fever are not available at present.)

In sexually transmitted diseases in men and women, azithromycin is indicated for the treatment of uncomplicated genital infections due to *Chlamydia trachomatis*. It is also indicated for the treatment of chancroid due to *Haemophilus ducreyi* and uncomplicated genital infections due to non-multiresistant *Neisseria gonorrhoeae*; concurrent infection with *Treponema pallidum* should be excluded.

RECOMMENDED DOSE

Zithrolide® Powder for Oral Suspension 200mg/5mL should be administered as a single daily dose. It can be taken with or without food.

Adults

For the treatment of sexually transmitted diseases caused by *Chlamydia trachomatis* and *Haemophilus ducreyi*, or susceptible *Neisseria gonorrhoeae*, the dose is 1000 mg as a single oral dose.

For patients who are allergic to penicillin and or cephalosporins, prescribers should consult local treatment guidelines.

Children

The maximum recommended total dose for any treatment is 1500 mg. In general, the total dose is 30 mg/kg. Treatment for pediatric streptococcal pharyngitis should be dosed at a different regiment.

The total dose of 30 mg/kg should be given as single daily dose of 10 mg/kg daily for 3 days, or given over 5 days with a single daily dose of 10 mg/kg on day 1, then 5 mg/kg on days 2-5.

As an alternative, treatment for acute otitis media can be given as a single dose of 30 mg/kg.

For pediatric Streptococcal pharyngitis, azithromycin given as a single dose of 10 mg/kg or 20 mg/kg for 3 days has been shown to be effective; however, a daily dose of 500 mg must not be exceeded.

Based on available clinical trials data comparing these two dosage regimens, similar clinical efficacy was observed but greater bacteriologic eradication was evident at the 20 mg/kg/day dose. However, penicillin is the usual drug of choice for the treatment of *Streptococcus pyogenes* pharyngitis, including prophylaxis of rheumatic fever.

For children weighing less than 15 kg, azithromycin suspension should be measured as closely as possible, and for children weighing 15 kg or more, administered according to the guide provided below.

Azithromycin Suspension 30 mg/kg Total Treatment Dose			
Weight (kg)	**3-Day Regimen**	**5-Day Regimen**	**Bottle Size (mg)**
<15	10 mg/kg once daily on Days 1-3	10 mg/kg on Day 1, then 5 mg/kg once daily on Days 2-5	600
15-25	200 mg (5 ml) once daily on Days 1-3	200 mg (5 ml) on Day 1, then 100 mg (2.5 ml) once daily on Days 2-5	600
26-35	300 mg (7.5 ml) once daily on Days 1-3	300 mg (7.5 ml) on Day 1, then 150 mg (3.75 ml) once daily on Days 2-5	900
36-45	400 mg (10 ml) once daily on Days 1-3	400 mg (10 ml) on Day 1, then 200 mg (5 ml) once daily on Days 2-5	1200
>45	Dose as per adults	Dose as per adults	1500

*Bottle sizes of 900mg, 1200mg & 1500mg are not available for this brand.

Special Populations:

Elderly

The dosage is same as in adult patients. Elderly patients may be more susceptible to the development of torsades de pointes arrhythmia than younger patients.

Renal Impairment

No dose adjustment is necessary in patients with mild to moderate renal impairment (GFR 10-80 ml/min). Caution should be exercised when azithromycin is administered to patients with severe renal impairment (GFR <10 ml/min).

Hepatic Impairment

The same dosage as in patients with normal hepatic function may be used in patients with mild to moderate hepatic impairment.

ROUTE OF ADMINISTRATION

Oral.

CONTRAINDICATIONS

The use of this product is contraindicated in patients with hypersensitivity to azithromycin, erythromycin, any macrolide or ketolide antibiotic, or to any excipient.

WARNINGS AND PRECAUTIONS

Hypersensitivity

In the event of severe acute hypersensitivity reactions, such as anaphylaxis, severe cutaneous adverse reactions (SCARs) [e.g. Stevens-Johnson Syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP)], Zithrolide® Powder for Oral Suspension 200mg/5mL should be discontinued immediately and appropriate treatment should be urgently initiated.

Hepatotoxicity

Since liver is the principal route of elimination for azithromycin, the use of azithromycin should be undertaken with caution in patients with significant hepatic disease. Abnormal liver function, hepatitis, cholestatic jaundice, hepatic necrosis, and hepatic failure has been reported, some of which have resulted in death. Discontinue azithromycin immediately if signs and symptoms of hepatic occur.

Infantile hypertrophic pyloric stenosis (IHPS)

Following the use of azithromycin in neonates (treatment up to 42 days of life), infantile hypertrophic pyloric stenosis (IHPS) has been reported. Parents and caregivers should be informed to contact their physician if vomiting or irritability with feeding occurs.

Ergot derivatives

In patients receiving ergot derivatives, ergotism has been precipitated by co-administration of some macrolide antibiotics. There are no data concerning the possibility of an interaction between ergot and azithromycin. However, because of the theoretical possibility of ergotism, azithromycin and ergot derivatives should not be co-administered.

Superinfection

As with any antibiotic preparation, observation for signs of super infection with non-susceptible organisms, including fungi is recommended.

Clostridium difficile-associated diarrhea

Clostridium difficile-associated diarrhea (CDAD) has been reported with the use of nearly all antibacterial agents, including azithromycin, and may range in severity from mild diarrhea to fatal colitis. Treatment with antibacterial agents alters the normal flora of the colon, leading to overgrowth of *C. difficile*.

C. difficile produces toxin A and B, which contribute to the development of CDAD. Hypertoxin-producing strains of *C. difficile* cause increased morbidity and mortality, as these infections can be refractory to antimicrobial therapy and may require colectomy. CDAD must be considered in all patients who present with diarrhea following antibiotic use. Careful medical history is necessary since CDAD has been reported to occur over 2 months after the administration of antibacterial agents.

Renal impairment

In patients with severe renal impairment (GFR <10 ml/min), a 33% increase in systemic exposure to azithromycin was observed.

Diabetes

Azithromycin 200mg/5ml powder for oral suspension:

Caution in diabetic patients: 5 ml of reconstituted suspension contains 4.55g of sucrose. Due to the sucrose content, this medicinal product is not indicated for persons with fructose intolerance (hereditary fructose intolerance), glucose-galactose malabsorption or saccharase-isomaltase deficiency.

Prolongation of the QT interval

Prolonged cardiac repolarization and QT interval, imparting risk of developing cardiac arrhythmia and torsades de pointes, have been seen in treatment with macrolides, including azithromycin. Prescribers should consider the risk of QT prolongation should be considered, which can be fatal when weighing the risks and benefits of azithromycin for at-risk groups including:

- Patients with congenital and documented QT prolongation.
- Patients currently receiving treatment with other active substances known to prolong QT interval, such as antiarrhythmics of Classes IA and III, antipsychotic agents, antidepressants, and fluoroquinolones.
- Patients with electrolyte disturbance, particularly in cases of hypokalemia and hypomagnesemia.
- Patients with clinically relevant bradycardia, cardiac arrhythmia, or cardiac insufficiency.
- Elderly patients, as they may be more susceptible to drug-associated effects on the QT interval.

Aspartame
Unsuitable for phenylketonurics as product contains aspartame.
Myasthenia gravis
Exacerbations of the symptoms of myasthenia gravis and new onset of myasthenia syndrome have been reported in patients receiving azithromycin therapy.

INTERACTIONS WITH OTHER MEDICAMENTS
Antacids
No effect on overall bioavailability was seen in simultaneous administration of antacid with azithromycin although peak serum concentrations were reduced by approximately 24%. In patients receiving both azithromycin and antacids, the medicinal products should not be taken simultaneously.

Cetirizine
Co-administration of a 5-day regimen of azithromycin with 20 mg of cetirizine at steady-state resulted in no pharmacokinetic interactions and no significant changes in QT interval.

Didanosine (Dideoxyinosine)
Co-administration of 1200 mg/day azithromycin with 400 mg/day didanosine did not affect the steady state pharmacokinetics of didanosine.

Digoxin and colchicine (P-gp substrates)
Concomitant administration of macrolide antibiotics, including azithromycin, with P-glycoprotein substrates such as digoxin and colchicine, has been reported to result in increased serum levels of the P-glycoprotein substrate. Therefore, if azithromycin and P-gp substrates such as digoxin are administered concomitantly, the possibility of elevated serum concentrations of the substrate should be considered. Clinical monitoring, and possibly serum digoxin levels during treatment with azithromycin and after its discontinuation is necessary.

Zidovudine
Single 1000 mg doses and multiple 1200 mg or 600 mg doses of azithromycin had little effect on the plasma pharmacokinetics or urinary excretion of zidovudine or its glucuronide metabolite. However, administration of azithromycin increased the concentrations of phosphorylated zidovudine, the clinically active metabolite, in peripheral blood mononuclear cells. The clinical significance of this finding its unclear, but it may be of benefit to patients.

Azithromycin does not interact significantly with hepatic cytochrome P450 system. It is not believed to undergo the pharmacokinetics drug interactions as seen with erythromycin and other macrolides. Hepatic cytochrome P450 induction or inactivation via cytochrome-metabolite complex does not occur with azithromycin.

Ergot
Due to the theoretical possibility of ergotism, the concurrent use of azithromycin with ergot derivatives is not recommended.

Atorvastatin
Co-administration of atorvastatin (10 mg daily) and azithromycin (500 mg daily) did not alter the plasma concentrations of atorvastatin (based on a HMG CoA-reductase inhibition assay). However, post-marketing cases of rhabdomyolysis in patients receiving azithromycin with statins have been reported.

Carbamazepine
No significant effect was observed on the plasma levels of carbamazepine or its active metabolite in patients receiving concomitant azithromycin.

Cimetidine
The effects of a single dose of cimetidine, given 2 hours before azithromycin, on the pharmacokinetics of azithromycin, no alteration of azithromycin pharmacokinetics was seen.

Coumarin-Type Oral Anticoagulants
Azithromycin did not alter the anticoagulant effect of a single 15 mg dose of warfarin administered. There have been reports received in the post-marketing period of potentiated anticoagulation subsequent to co-administration of azithromycin and coumarin-type oral anticoagulants. Although a causal relationship has not been established, consideration should be given to the frequency of monitoring prothrombin time when azithromycin is used in patients receiving coumarin-type oral anticoagulants.

Cyclosporin
Administration of a 500 mg/day oral dose of azithromycin for 3 days and then a single 10 mg/kg oral dose of cyclosporin results in cyclosporin C_{max} and AUC_{0-5} to be significantly elevated. Consequently, caution should be exercised before considering concurrent administration of these drugs. If co-administration of these drugs is necessary, cyclosporin levels should be monitored and the dose adjusted accordingly.

Efavirenz
Co-administration of a 600 mg single dose of azithromycin and 400 mg efavirenz daily for 7 days did not result in any clinically significant pharmacokinetic interactions.

Fluconazole
Co-administration of a single dose of 1200 mg azithromycin did not alter the pharmacokinetics of a single dose of 800 mg fluconazole. Total exposure and half-life of azithromycin were unchanged by the co-administration of fluconazole, however, a clinically insignificant decrease in C_{max} (18%) of azithromycin was observed.

Indinavir
Co-administration of a single dose of 1200 mg azithromycin had no statistically significant effect on the pharmacokinetics of indinavir administered as 800 mg three times daily for 5 days.

Methylprednisolone
No significant effect on the pharmacokinetics of methylprednisolone.

Midazolam
Co-administration of azithromycin 500 mg/day for 3 days did not cause clinically significant changes in the pharmacokinetics and pharmacodynamics of a single 15 mg dose of midazolam.

Nelfinavir
Co-administration of azithromycin (1200 mg) and nelfinavir at steady state (750 mg three times daily) resulted in increased azithromycin concentrations. No clinically significant adverse effects were observed and no dose adjustment is required.

Rifabutin
Co-administration of azithromycin and rifabutin did not affect the serum concentrations of either medicinal product.

Neutropenia was observed in subjects receiving concomitant treatment of azithromycin and rifabutin. Although neutropenia has been associated with the use of rifabutin, a causal relationship to combination with azithromycin has not been established.

Sildenafil
There was no evidence of an effect of azithromycin (500 mg daily for 3 days) on the AUC and C_{max} of sildenafil or its major circulating metabolite.

Terfenadine
No evidence of an interaction between azithromycin and terfenadine. There have been rare cases reported where the possibility of such an interaction could not be entirely excluded; however, there was no specific evidence that such an interaction had occurred.

Theophylline
No evidence of interaction when azithromycin and theophylline are co-administered.

Triazolam
Co-administration of azithromycin 500 mg on Day 1 and 250 mg on Day 2 with 0.125 mg triazolam on Day 2 had no significant effect on any of the pharmacokinetic variables for triazolam compared to triazolam and placebo.

Trimethoprim/sulfamethoxazole
Co-administration of trimethoprim/sulfamethoxazole (160 mg/800 mg) for 7 days with azithromycin 1200 mg on Day 7 had no significant effect on peak concentrations, total exposure or urinary excretion of either trimethoprim or sulfamethoxazole. Azithromycin serum concentrations were similar to those seen in other studies.

PREGNANCY AND LACTATION
Pregnancy
Based on the available animal reproduction studies which performed at doses up to moderately maternally toxic dose concentrations, no evidence of harm to the fetus due to azithromycin was found. There are, however, no adequate and well-controlled studies in pregnant women. Because animal reproduction studies are not always predictive of human response, azithromycin should be used during pregnancy only if clearly needed.

Lactation
Limited information available from published literature indicates that azithromycin is present in human milk at an estimated highest median daily dose of 0.1 to 0.7 mg/kg/day. No serious adverse effects of azithromycin on the breast-fed infants were observed.

A decision must be made whether to discontinue breast-feeding or to discontinue/abstain from azithromycin therapy taking into account the benefit of breast-feeding for the child and the benefit or therapy for the woman.

Fertility
Reduced pregnancy rates were noted following administration of azithromycin. The relevance of this finding to humans is unknown.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES
There is no evidence to suggest that azithromycin may have an effect on the patient's ability to drive or operate machinery.

ADVERSE EFFECTS
Azithromycin is well tolerated with a low incidence of side effects.

Based on the available clinical trials, the following undesirable effects have been reported:

Blood and Lymphatic System Disorders
Transient episodes of mild neutropenia.

Ear and Labyrinth Disorders
Hearing impairment (including hearing loss, deafness and/or tinnitus) has been reported in some patients receiving azithromycin. Many of these have been associated with prolonged use of high doses in investigational studies. In those cases where follow-up information was available, the majority of these events were reversible.

Gastrointestinal Disorders
Nausea, vomiting, diarrhea, loose stools, abdominal discomfort (pain/cramps), and flatulence.

Hepatobiliary Disorders
Abnormal liver function.

Skin and Subcutaneous Tissue Disorders
Allergic reactions including rash and angioedema.

General Disorders and Administration Site Conditions
Local pain and inflammation at the site of infusion.

In post-marketing experience, the following additional undesirable effects have been reported:

Infections and Infestations
Moniliasis and vaginitis.

Blood and Lymphatic System Disorders
Thrombocytopenia.

Immune System Disorders
Anaphylaxis (rarely fatal).

Metabolism and Nutrition Disorders
Anorexia.

Psychiatric Disorders
Aggressive reaction, nervousness, agitation, and anxiety.

Nervous System Disorders
Dizziness, convulsions, headache, hyperactivity, hypoesthesia, paresthesia, somnolence, and syncope.

There have been rare reports of taste/smell perversion and/or loss.

Ear and Labyrinth Disorders
Deafness, tinnitus, hearing impaired, and vertigo.

Cardiac Disorders
Palpitations and arrhythmias including ventricular tachycardia have been reported. There have been rare reports of QT prolongations and torsades de pointes.

Vascular Disorders
Hypotension.
Gastrointestinal Disorders
Vomiting/diarrhea (rarely resulting in dehydration), dyspepsia, constipation, pseudomembranous colitis, pancreatitis, rare reports of tongue discoloration and infantile hypertrophic pyloric stenosis.

Hepatobiliary Disorders
Hepatitis and cholestatic jaundice have been reported, as well as rare cases of hepatic necrosis and hepatic failure, which have resulted in death.

Skin and Subcutaneous Tissue Disorders
Allergic reactions including pruritus, rash, photosensitivity, edema, urticaria, and angioedema. Frequency not known: severe cutaneous adverse reactions (SCARs) including Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN), drug reaction with eosinophilia and systemic symptoms (DRESS) & acute generalised exanthematous pustulosis (AGEP).

Musculoskeletal and Connective Tissue Disorders
Arthralgia.

Renal and Urinary Disorders
Interstitial nephritis and acute renal failure.

General Disorders and Administration Site Conditions
Asthenia, fatigue, and malaise.

OVERDOSE AND TREATMENT
Adverse events experienced in higher than recommended doses were similar to those seen at normal doses. In the event of overdosage, general symptomatic and supportive measures indicated as required.

STORAGE CONDITION
Dry powder (unopen bottle): Store below 30°C.
Once reconstituted with water, store in refrigerator at 2-8°C.

INSTRUCTION FOR USE AND HANDLING
Tap the bottle to loosen the powder. Add 8 ml of water to the bottle and shake well. Shake immediately prior to use.

For children weighing less than 15 kg, the suspension should be measured as closely as possible. For children weighing 15 kg or more, the suspension should be administered using an appropriate measuring device.

SHELF LIFE
Dry powder (unopen bottle): Product should not be used beyond the expiry date imprinted on the product packaging.

Once reconstituted with water, the suspension has a shelf life of 5 days.

PACK SIZE
The powder for oral suspension is packed in HDPE bottle.
Packs of powder equivalent to 600 mg azithromycin.
Content of the bottle after reconstitution: 15 ml.

REGISTRATION NUMBER
MAL23076010AZ

CONTROLLED MEDICINE
UBAT TERKAWAL

KEEP MEDICINES OUT OF REACH OF CHILDREN
JAUHI UBAT-UBATAN DARI KANAK-KANAK

For further information, please consult your doctor or your pharmacist.

Revision number : 00
Revision date : 30 May 2023

Lt-234.00 Product Registration Holder and Manufacturer
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